

Master 09 - Evidence Dossier, Sources, Claims Register & Caveats

Audience: Dean, OTOS advisors, evidence reviewers, commissioners, Public Health, strategic partners, document owners.

Purpose: Control claims, sources, caveats and approved wording across the OTOS copy family.

Status: PENDING EVIDENCE CHECK.

Priority: P1.

Review needed from: Evidence reviewer; clinical reviewer for clinical-adjacent claims; Dean for final copy tone.

Source basis: Master 09 source PDF, Evidence Dossier PDF, START report, Programme Lock, Programme Plan, START replacement pack.

A4 REVIEW PAGE 1 - What this document is

This is the claims-control document for the OTOS Continuity™ copy family.

It exists so official documents, PDFs, partner briefs, investor materials, commissioner submissions, strategic pharma materials and future visual copy can use the same evidence base without drifting into unsafe claims.

It does four jobs:

1. holds the approved numbers;
2. separates published evidence from OTOS modelling;
3. records claim-safe wording;
4. shows what must not be claimed.

The rule is simple:

If a number, quote or claim appears in the official pack, it must be traceable here or deliberately labelled as modelled / indicative / TBC.

Evidence categories

Use five evidence categories consistently:

1. **Published evidence** - peer-reviewed studies, NICE guidance, RCPsych guidance, national reports and published evidence reviews.

2. **Local service data** - CPFT / NHS / Cambridgeshire & Peterborough figures or partner data, pending local validation.
3. **OTOS modelling** - scenario estimates, cost-chain assumptions and planning calculations, never guaranteed.
4. **Policy alignment** - direction-of-travel fit with NHS / DHSC / public-sector strategy; alignment, not endorsement.
5. **Founder arc / clinical receipt** - lived pathway and documented clinical wording; design evidence, not universal proof.

Product truth spine

Terminology lock

Use **user journey** / **person's journey** / **service journey** instead of **pathway** wherever possible. The reason is important: OTOS is not claiming that a single integrated pathway already exists. OTOS provides continuity infrastructure around the user's real journey across separate services, gaps, handoffs and waiting periods.

Category

NIA - Neurologically Informed Addiction - is working terminology for addiction, relapse or repeat crisis where untreated or inadequately treated ADHD may be a material driver.

NIA is not a diagnosis.

Intervention

The intervention tests whether the OTOS Continuity Management Layer and user Continuity Thread can support a suspected ADHD-driven addiction cohort:

- earlier recognition of suspected ADHD-driven addiction;
- ADHD clinical-route / next-step status where agreed;
- qualified ADHD assessment and treatment where clinically indicated and clinically owned;
- authorised partner activity through the Continuity Management Layer;
- a Continuity Thread following the user's whole-person journey over time through agreed non-clinical events;
- recovery work, waiting-well support, service handoffs, behavioural reinforcement and habit-stacking practice having a better chance to land;
- evidence of what happened;
- a decision to continue, continue with changes, rebuild, pause or stop.

Infrastructure

OTOS Continuity™ provides the non-clinical Continuity Management Layer:

- tokenised user Continuity Thread;
- consent records;
- touchpoint logging;
- handoff receipts;
- partner-approved SMS check-ins;
- delivery / failure / reply / silence evidence;
- drift / silence visibility;
- human review;
- evidence export.

Layer = the non-clinical infrastructure partners and the user access. Thread = the user's whole-person journey over time, built from agreed non-clinical events. Service lines = each partner's own records, communication channels, service work and responsibilities.

OTOS does not diagnose, prescribe, treat, replace services, make clinical decisions or write into NHS clinical records.

A4 REVIEW PAGE 2 - START evidence-control section

START handling

START should be described as a **real-world dual-diagnosis service-journey signal**, not as a standalone medication proof.

The French START study was a retrospective observational review of people already in addiction treatment, with addictive-disorder outcomes assessed six months after ADHD diagnosis.

The headline figure - 61.3% showing improvement - is important, but it sits inside a routine-care pathway where diagnosis, care engagement, treatment timing, methylphenidate initiation and follow-up all interacted.

In 29% of cases, methylphenidate initiation was delayed. That means the six-month outcome window was not a uniform medication-exposure window.

The safe conclusion is:

START helps justify testing whether ADHD can be identified, clinically routed, treated where appropriate, followed through and monitored alongside addiction outcomes. It does not prove that medication alone caused the improvement, does not prove that methylphenidate directly caused all outcomes, and does not predict that OTOS will achieve 61.3% improvement.

Approved START short line

START helps justify the test. It is a real-world dual-diagnosis service-journey signal, not an OTOS outcome and not a medication-only proof. OTOS must generate its own local evidence.

What START supports

START can be used to support:

- identifying ADHD inside addiction-treatment populations;
- clinically confirming ADHD rather than assuming it;
- treating ADHD where clinically indicated and medically supervised;
- monitoring ADHD and addiction outcomes together;
- following people through the user journey rather than losing them between diagnosis, treatment, addiction support and recovery work;
- designing a UK local test that captures real-world user-journey evidence.

What START must not be used to claim

Do not claim:

- OTOS will achieve 61.3% improvement;
- medication alone caused the improvement;
- methylphenidate directly caused all outcomes;
- medication cures addiction;
- START proves OTOS works;
- START can be copied directly into the UK without adaptation;
- a First-50 test can prove national clinical outcomes.

A4 REVIEW PAGE 3 - Claims register control rows

Claim / phrase	Classification	Safe version	Notes
"START showed a 61.3% reduction."	Avoid	START reported that 61.3% of patients showed improvement in addictive disorders six months after ADHD diagnosis, in a retrospective addiction-treatment cohort.	Evidence-controlled sections only.
"START proves medication improves addiction."	Avoid	START suggests that identifying ADHD inside addiction-treatment populations and treating it within routine care can coincide with	Not a causal medication-only claim.

Claim / phrase	Classification	Safe version	Notes
		improvement in addictive outcomes.	
"Methylphenidate caused 61.3% improvement."	Banned	The START material does not support a medication-only causal reading; diagnosis, treatment timing, engagement, addiction care and follow-up interacted.	Must not appear externally.
"Medication fixes addiction."	Banned	ADHD treatment may open a learning window where clinically indicated; recovery work, addiction support, CBT, waiting-well support and continuity still need to land.	Use across all docs.
"OTOS treats addiction."	Banned	OTOS is non-clinical continuity infrastructure. Addiction treatment remains with partner services.	Core boundary.
"OTOS diagnoses ADHD / NIA."	Banned	OTOS may help partners identify a suspected ADHD-driven addiction pattern. ADHD diagnosis remains with qualified clinical teams. NIA is working terminology, not a diagnosis.	Core boundary.
"OTOS creates a shared clinical record."	Banned	OTOS provides shared continuity infrastructure, not a shared clinical record. The Continuity Thread follows the user journey through agreed non-clinical events.	Core boundary.
"OTOS will save money."	Avoid unless modelled	OTOS is designed to generate local evidence about whether earlier recognition and continuity may reduce avoidable crisis and late spend.	Local evidence pending.
"First-50 proves the model."	Avoid	The First-50 test produces decision-grade local evidence, not national proof.	Use "continue / continue with changes / rebuild / pause / stop".
"Medicine-plus-Continuity."	Approved with placement limit	Use sparingly in strategic / NIA / pharma-category material. In most NHS, commissioner, partner and operating copy, use suspected ADHD-driven addiction intervention, Continuity	Keep clinical ownership explicit.

Claim / phrase	Classification	Safe version	Notes
		Management Layer and Continuity Thread language.	
"ADHD treatment may open the learning window."	Approved with caveat	Use where clinically indicated and clinically owned; do not imply suitability for everyone.	Strong mechanism line.
"OTOS keeps the window open."	Avoid / use only as metaphor with care	Prefer: the Continuity Management Layer helps authorised partners keep recovery, waiting-well support and behavioural reinforcement visible and repeated while clinical decisions remain clinically owned.	Best in Science of Sticking / narrative.
"Decision-grade local evidence."	Approved	Use for First-50 evidence posture.	Not national proof.

A4 REVIEW PAGE 4 - Master claims guardrails

Approved language

Use:

- suspected ADHD-driven addiction within the user journey;
- NIA as working terminology, not diagnosis;
- where clinically indicated;
- qualified clinical ADHD route;
- ADHD clinical-route / next-step status where agreed;
- suspected ADHD-driven addiction intervention;
- Continuity Management Layer;
- tokenised user Continuity Thread;
- shared continuity infrastructure, not shared clinical record;
- SMS delivery / failure / reply / silence evidence;
- human review;
- decision-grade local evidence;
- START as real-world dual-diagnosis service-journey signal;
- continue, continue with changes, rebuild, pause or stop;
- clinical teams make clinical decisions;

- partner teams retain service responsibility;
- the Continuity Management Layer surfaces agreed continuity signals.

Avoid or ban

Do not use:

- medication cures addiction;
- stimulant treatment solves addiction;
- OTOS treats addiction;
- OTOS diagnoses ADHD;
- OTOS diagnoses NIA;
- OTOS prescribes or manages medication;
- OTOS makes clinical decisions;
- OTOS provides therapy;
- OTOS provides crisis care;
- OTOS safeguards independently;
- shared clinical record;
- full pathway integration;
- guaranteed relapse reduction;
- guaranteed savings;
- START-level replication;
- 61.3% expected outcome;
- national proof from the first test;
- Phase 2 or rollout is guaranteed.

Evidence citation placeholders before external use

Before this document is externally circulated, insert checked citations for:

- START French study;
- NICE ADHD guidance / combination treatment references;
- ADHD medication and substance-misuse risk evidence;
- ADHD Taskforce economic-cost claim;
- alcohol-harm / public-cost figures;
- any local CPFT / C&P / partner figures;

- founder clinical-letter quote, where used.

Collected evidence sweep — retained data to fold into Master 09

Status: Working evidence-control material imported from the curated laptop evidence sweep. These rows are not final citation copy yet. They should be used to control wording, prioritise validation, and feed the Evidence PDF / claims register / document-family edits.

Priority validation jobs

1. **START / 61.3%** — confirm study title, authors, year, source, population, outcome definition, six-month window and caveats.
2. **ADHD / addiction overlap** — confirm prevalence range and source.
3. **NICE / RCPsych / ADHD-with-substance-use guidance** — confirm exact wording and safe interpretation.
4. **Local denominator / cohort modelling** — confirm 4,175 denominator, 800–900 model, and label the 800–900 figure as OTOS modelling, not CPFT-reported.
5. **Cost / public-health figures** — keep externally sourced public-health / economic evidence only. Keep old OTOS build-budget estimates out of the evidence page.
6. **12-week active support / 3- and 6-month follow-up** — treat 12 weeks as the active operating window, not the full evidence horizon.
7. **User-journey terminology** — replace “pathway” where possible. OTOS wraps around the user journey; it does not claim a single joined-up pathway already exists.

Evidence / stat candidates to control

Item	Claim / stat	Status	Approved-use caveat / action
START 61.3%	START study — France, 2025 — 61.3% improvement signal in addictive-disorder outcomes at six months after ADHD diagnosis / appropriate treatment.	Likely reusable after exact citation check.	Do not present as an OTOS outcome, medication-only proof, or a prediction that OTOS will achieve 61.3%. Find original START paper/report and capture full citation, method, population and exact outcome wording.
START six-month wording	At six-month follow-up, 61.3% showed improvement in addictive-disorder outcomes.	Likely reusable after exact citation check.	Verify whether “clinically significant improvement” is exactly supported and define the outcome measure if available.

Item	Claim / stat	Status	Approved-use caveat / action
START design description	START followed adults entering addiction treatment who were also screened and treated for ADHD.	Likely reusable after exact citation check.	Confirm setting, cohort, retrospective / observational design and treatment details.
START / BMJ / RCPsych stack	START, BMJ / Southampton-Karolinska and RCPsych CR235 form part of the evidence-depth stack for ADHD, addiction and treatment-risk discussion.	Internal source map.	Convert into formal citation rows. Do not use as a standalone external claim.
NICE NG87	NICE NG87 supports ADHD assessment and treatment in active substance-misuse contexts.	Likely reusable after exact wording check.	Check exact NICE wording. Do not imply NICE endorses OTOS.
Substance use and ADHD treatment	Co-occurring substance use is not necessarily a contraindication to ADHD diagnosis or treatment.	Needs citation.	Tie to exact NICE / RCPsych wording and include risk-management conditions.
RCPsych CR235	RCPsych 2022 / CR235 indicates benefit may outweigh risk when treating ADHD in patients with substance use disorder, including ADHD symptom and substance-use outcomes.	Needs citation.	Check exact CR235 wording and section/page. High-risk if misquoted.
ADHD in addiction cohorts	Prevalence of ADHD in addiction cohorts: 23–45%.	Needs citation.	Identify original prevalence source(s), setting and whether the range applies to SUD, alcohol-use disorder, addiction-treatment cohorts or broader SUD cohorts.
Alcohol-use disorder / ADHD	Adults with alcohol-use disorder who meet ADHD criteria may vary from 20% to 50%, depending on study population.	Needs citation.	Use carefully. Consider keeping in evidence-controlled materials rather than first-read copy.
ADHD–SUD severity	ADHD–SUD comorbidity is associated with more severe addiction patterns, earlier onset of use,	Needs citation.	Locate review / study citations. Avoid presenting as universal for every participant.

Item	Claim / stat	Status	Approved-use caveat / action
	higher polysubstance abuse rates, increased relapse risk and intensified ADHD symptoms.		
ADHD and addiction risk	ADHD is linked to higher risk of substance-use disorders and behavioural addictions.	Needs citation.	Attach primary sources / systematic reviews before external use.
NHS ADHD Taskforce — £17bn	NHS ADHD Taskforce / April 2025 frames unsupported ADHD as linked to educational failure, unemployment, crime, substance misuse, suicide and mental / physical illness, with avoidable economic costs of at least £17bn.	High-priority citation check.	Verify exact wording, date and document title. Separate national economic burden from OTOS outcome claims.
NHS ADHD Taskforce — public health	Undiagnosed or unsupported ADHD is framed as a major under-addressed public-health issue.	Needs citation.	Confirm exact wording: “undiagnosed”, “unsupported”, “unmet need” or other phrase.
NHS 10 Year Health Plan alignment	Strategic fit with hospital to community, analogue to digital, sickness to prevention.	Needs citation.	Use as policy alignment only, not endorsement.
NHS Community Hubs	Potential alignment with 250 NHS Community Hubs / community health infrastructure.	Needs citation / optional.	Verify the 250 figure and decide whether this belongs in strategic appendix only.
DHSC addiction technology / fragmentation	DHSC addiction-healthcare goals include investment in addiction technology addressing structural fragmentation.	Needs citation.	Find DHSC source and quote exact wording.
OTOS fragmentation alignment	OTOS aligns with reducing fragmentation in addiction and mental-health service journeys.	Internal positioning.	Say “aligns with”; never imply endorsement.
Local denominator	Adults in structured addiction treatment across	Needs citation.	Confirm source, year, geography and whether

Item	Claim / stat	Status	Approved-use caveat / action
	Cambridgeshire and Peterborough: 4,175.		NDTMS / local authority / public-health data.
Local risk cohort model	OTOS planning estimate: 800–900 local ADHD + addiction risk cohort, based on 4,175 adults in treatment and ADHD-prevalence assumptions.	Internal planning only.	Label clearly: OTOS modelling estimate; not CPFT-reported number.
800–900 caveat	The 800–900 figure is not a CPFT-reported number.	Claims-control wording.	Preserve caveat wherever the model appears.
Data-source caveat	Adult addiction-treatment data are stronger than adult ADHD service data.	Internal modelling caveat.	Use internally unless supported by a specific data-source comparison.
Geography caveat	Addiction data are often local-authority based, while ADHD service data are NHS-provider or national-proxy based.	Internal modelling caveat.	Use to explain why local modelling is imperfect.
CPFT waiting list	Significant and growing ADHD waiting list within CPFT.	Needs local validation.	Find current CPFT / public source or remove from external copy.
First-50 cohort	Pre-pilot demonstrator cohort: up to 50 adults.	Internal design assumption.	Use as design choice, not evidence finding.
12-week operating window	12 weeks is enough to test touchpoints, engagement, handoff receipts and drift signals.	Internal planning only / needs reframing.	Reframe as 12 weeks active support / operating window, with 3- and 6-month follow-up evidence where consent, governance and data-sharing allow.
Modelled avoidable demand	OTOS scenario modelling estimate: approximately £18m modelled avoidable annual demand / opportunity.	Internal planning only.	Not a guaranteed saving. Keep out of first-read external copy unless fully sourced and caveated.
Scenario calculator	Example: $4,175 \times 25\%$ ADHD prevalence $\times 25\%$ effect $\times$ £40,070 annual burden = £10,455,766 gross annual saving.	Internal modelling only.	Modelling appendix only. Do not use as promised saving.
Turning Point / LSE	£4.07 return per £1 invested in addiction continuity / addiction interventions.	Needs citation.	Find original source and check exact programme. Do not imply direct applicability to OTOS.

Item	Claim / stat	Status	Approved-use caveat / action
Big Health Sleepio benchmark	Sleepio evaluated at roughly £55–65 per patient for a full course in early implementations and shown to be cost-saving within one year.	Needs citation / optional.	Use only as comparator economics if needed; otherwise appendix only.
Founder clinical letter	Psymlicity clinical letter records ADHD complicated by excess alcohol use used to self-medicate inadequately managed ADHD symptoms, improved response to Elvanse, and asks for faster NHS access to effective treatment.	Private / case-specific.	Use only in private evidence archive / founder-story guardrail. Not generalisable and not for broad circulation without consent/legal/clinical review.

Evidence-discipline rules imported from the sweep

- Every serious claim should be traceable.
- Every number must carry a label: published evidence, local service data, OTOS modelling, policy alignment, or founder arc / clinical receipt.
- Separate observed, modelled, proxy and assumption figures.
- START, BMJ / Southampton-Karolinska, RCPsych CR235 and other sources should be used as evidence depth, not guaranteed outcomes for OTOS.
- Use RCPsych CR235, NICE / NHS guidance, NHS Independent ADHD Taskforce context, ADHD-and-substance-use overlap evidence and peer-reviewed outcome studies as the evidence-depth hierarchy.
- Keep build-cost figures out of this evidence dossier unless they are in a separate development-costing / supplier-costing section.
- Remove old or unsupported build-budget figures such as £90k, £90k–£150k and £91,520 from evidence claims.

Additional NHS / local evidence files to review

Status: Laptop / Drive evidence intake now captured for Master 09. These files should be treated as potentially useful source material, not approved evidence until reviewed and converted into citation-controlled rows. This page is now the holding and control point for the extra NHS / ADHD / addiction / digital-governance content supplied by Dean.

Files supplied for review

- OTOS_Evidence_Quote_Bank_v0_2.csv
- cpft_truth_table.csv
- england_trusts.csv
- _OD13 Addiction Support Research.md
- 2025-09-25__New_ADHD_Taskforce_findings_to_inform_strategy__68d512f1.md
- 2025-09-25__NHS__68d4c26f.md
- 2025-11-25__EYE10-NHS__692640ed.md
- 2025-11-26__UK_ADHD_SUD_Economic_Impact__69267398.md
- 2025-12-04__NHS_Minister_Pack_Build__6931e3ee.md
- 2025-12-13__EYE18__ADHD_UK__Positioning_2026__693d50e2.md
- Evidence-Review-Cambridgeshire-Drugs-and-Alcohol-Needs-Assessment-2024-25.docx
- NHS Trusts ADHD_Addiction Regional Report (Summary).docx
- cr235-adhd-in-adults---good-practice-guidance.pdf
- Deep Research Report on the French START Study and Stimulant Treatment of ADHD With Co-occurring Add.pdf
- OTOS Continuity deep research_ quote bank on ADHD, addiction, comorbidity, and integrated care.pdf
- Deep research report_ quotes and evidence linking ADHD and addiction for OTOS Continuity.pdf
- Digital Mental-Health Startup Challenges and Failures (2020–2025).pdf
- fit-for-the-future-10-year-health-plan-for-england-executive-summary.pdf
- NHS 10 Year Plan Guide 2025-V2.pdf
- missing gems.pdf
- NHS Data Integration & Reporting Standards for Digital Therapy Tools.pdf
- NHS Digital-Therapy Procurement & Economics.pdf
- OTOS Continuity deep research_ quote bank on ADHD, addiction, comorbidity, and integrated care.pdf
- Public-record review of NHS ADHD and addiction-service collaborations.pdf
- Outcomes of Digital and Hybrid Addiction Recovery Programs.pdf
- PRN02228-report-of-the-independent-adhd-taskforce-part-2.pdf
- PRN02031-interim-report-of-the-independent-adhd-taskforce-part-1.pdf
- Psychotherapeutic-assessment-approaches-in-diagnostic-assessment-of-ADHD-for-adults-with-SMD-presentation-slides.pdf

- `nhs_docs from drive.zip`

How these should be used

- Pull out exact NHS / CPFT / national-policy data points, quotes and figures that can strengthen the Evidence Dossier, Evidence PDF and commissioner-facing documents.
- Separate national NHS / Taskforce findings from local CPFT / Cambridgeshire and Peterborough claims.
- Use the `cpft_truth_table.csv` material to check what can and cannot safely be said about CPFT, local ADHD waiting-list position, local service structure and local evidence gaps.
- Use `england_trusts.csv` only as background / mapping material unless a specific trust-level claim is needed.
- Use the quote bank only after each quote has a clear source, status, caveat and approved wording.
- Preserve the user-journey terminology lock: use these sources to support the case that OTOS wraps around the person's journey across fragmented services, not that a single joined-up pathway already exists.

New validation task

- ☐ Review Dean's additional NHS / CPFT / quote-bank files and convert usable evidence into controlled rows with source, figure / quote, status, caveat and approved wording.
- ☐ Review the second batch of NHS / ADHD-SUD / Minister Pack / Cambridgeshire Drug and Alcohol Needs Assessment files and extract any stronger national, regional or local figures into the controlled evidence table.
- ☐ Review CR235 in detail and convert the ADHD / substance-misuse, assessment, medication-monitoring, environmental-modification and service-design points into source-controlled rows.
- ☐ Review the START deep-research report against the original START source before relying on any interpretation.
- ☐ Review digital mental-health startup failures as a governance / resilience appendix only, not as proof of OTOS effectiveness.
- ☐ Review the NHS 10 Year Plan files for exact wording on the three shifts and neighbourhood / community infrastructure alignment.
- ☐ Review the Independent ADHD Taskforce Part 1 and Part 2 reports for exact national wording on unmet need, economic / social costs, service fragmentation and implementation recommendations.

- ☐ Review NHS digital-therapy data integration, reporting, procurement and economics files for governance / buyer-readiness rows, keeping procurement claims separate from clinical-effectiveness claims.
- ☐ Review public-record NHS ADHD / addiction collaboration material for any genuine precedent rows, clearly separating public-record evidence from inferred opportunity.
- ☐ Review digital / hybrid addiction recovery outcomes evidence for comparator rows only; do not imply direct OTOS outcomes without local evidence.
- ☐ Review the new OTOS quote bank and “missing gems” file for candidate citations, then trace each useful quote back to its original source before approval.
- ☐ Review ADHD diagnostic-assessment / SMD slides for clinical-assessment context only; do not treat slides as policy unless source status is verified.
- ☐ Unpack and review `nhs_docs from drive.zip`; deduplicate against the files already listed above, then extract any missing NHS / Taskforce / 10 Year Plan / local-service evidence into controlled rows.

CR235 controlled evidence rows — first pass

Source: Royal College of Psychiatrists (2023), College Report CR235, *Attention deficit hyperactivity disorder (ADHD) in adults: Good practice guidance*.

Status: Strong source for clinical-boundary, service-design and ADHD-with-substance-misuse wording. Use as guidance / policy-aligned evidence, not as an endorsement of OTOS.

Item	Claim / quote-safe wording	Status	Approved-use caveat / action
Adult ADHD underdiagnosis / unmet need	CR235 states that adult ADHD continues to be underdiagnosed, identification is increasing, and there remains a large unmet need.	Likely reusable with citation.	Use as UK-relevant adult ADHD service-context evidence. Do not imply this is Cambridgeshire-specific unless local data confirms it.
Comorbidity is common	CR235 states there are high rates of comorbidity between ADHD and other mental-health / neurodevelopmental conditions, and that assessment should consider overlap to avoid misdiagnosis.	Likely reusable with citation.	Supports OTOS caution that NIA is not a diagnosis and that clinical assessment remains with qualified teams.
Assessment quality	CR235 says good-quality adult ADHD assessment takes time, is ideally multidisciplinary, and should draw on multiple	Likely reusable with citation.	Use to reinforce that OTOS does not diagnose ADHD; OTOS may support continuity

Item	Claim / quote-safe wording	Status	Approved-use caveat / action
	information sources, including third-party / longitudinal information where possible.		and evidence capture around a clinically owned process.
Management is multimodal	CR235 describes ADHD management as involving pharmacological, psychological, educational and skills-based interventions, plus adaptations to home, work and community environments.	Likely reusable with citation.	Supports Medicine-plus-Continuity as a broad support model, but do not imply OTOS provides treatment.
Environment first	CR235 states that environmental adaptation is a first-line approach and should be implemented and reviewed prior to and alongside medication treatments.	Likely reusable with citation.	Useful support for “medicine opens the window; recovery and practical support still need to land.”
Substance misuse overlap	CR235 states that substance misuse, mostly involving alcohol and cannabis, may be twice as common among individuals with ADHD as in the general population and tends to begin earlier.	Likely reusable with citation.	High-value evidence row. Quote carefully and cite CR235 section 11.1 / underlying Breyer et al. source if possible.
Self-medication	CR235 notes that individuals with untreated ADHD may describe self-medicating with illicit substances, and some report paradoxical calming effects from stimulant drugs such as cocaine and amphetamines.	Likely reusable with citation / clinically sensitive.	Use only in evidence-controlled or clinical-adjacent materials. Avoid sensational wording.
ADHD as risk factor for substance misuse	CR235 states that ADHD is a recognisable risk factor for early-onset substance misuse and, if not adequately treated, increased likelihood of relapse and chronic patterns with comorbid psychiatric symptoms.	Likely reusable with citation / high priority.	Strong support for the OTOS rationale, but phrase as risk / association, not deterministic causation.
Treatment and SUD risk	CR235 states there is little evidence of patients becoming dependent when prescribed stimulants are used appropriately, and treating	Likely reusable with citation / clinical review required.	Must remain clinically owned. Do not convert into “medication prevents addiction” or

Item	Claim / quote-safe wording	Status	Approved-use caveat / action
	ADHD may reduce lifetime risk of developing substance-use disorder.		"medication treats addiction."
Controlled misuse caveat	CR235 says controlled misuse should not completely preclude treatment, although careful monitoring is required; chaotic drug / alcohol misuse makes stimulant medication difficult to manage safely and not routinely recommended.	Likely reusable with citation / clinical review required.	Excellent boundary wording for "not excluded, but risk-managed."
Joint working with addiction services	CR235 states significant comorbid substance misuse should be managed jointly between general adult and addiction services in accordance with local protocols.	Likely reusable with citation.	Supports OTOS "partner services retain responsibility; OTOS wraps around the user journey."
Post-diagnostic support	CR235 states that psychoeducation and post-diagnostic support should be a component of care after diagnosis.	Likely reusable with citation.	Supports continuity / waiting-well / support-to-land framing, not OTOS as therapy.
Service design	CR235 states future service models should be stepped, matched, multidisciplinary, delivered in partnership and co-constructed with lived-experience voices.	Likely reusable with citation.	Good fit for OTOS partner model. Avoid "pathway" unless quoting CR235 or describing a clinically owned ADHD route.
Accessible connected services	CR235 links adult ADHD service development to accessible, connected services and notes current service configuration / resources were not meeting patient-group needs.	Likely reusable with citation.	Use as policy / service-design context, not proof that OTOS is the answer.
Flexible contact / reminders	CR235 notes rigid access policies may not suit this population and that technology such as text-message reminders or remote consultation may help with attendance.	Likely reusable with citation.	Strong support for SMS check-ins / continuity signals, but frame as alignment with need, not clinical outcome proof.

Item	Claim / quote-safe wording	Status	Approved-use caveat / action
Monitoring medication	CR235 states weight, blood pressure, heart rate and side-effect monitoring should be completed during titration / review and every six months once stabilised.	Likely reusable with citation.	Use only to define what remains clinical / prescribing responsibility; OTOS does not monitor medication clinically unless explicitly authorised and clinically governed.

CR235 safe synthesis line

CR235 supports the need for clinically owned, multidisciplinary adult ADHD assessment and management, recognises high comorbidity and substance-misuse overlap, and supports joined-up, flexible, partnership-based service design. For OTOS, the safe use is: **clinical teams assess and treat; OTOS provides non-clinical continuity infrastructure around the user journey, with evidence capture, consent, touchpoints, handoff visibility and partner communication support.**

Evidence rows ready for sign-off — added from supplied documents

Status: These are extracted claim-ready rows from the supplied documents. They still need Dean / evidence / clinical / governance tick-box sign-off before use in external copy.

Sign-off boxes — remaining for Master 09

- ☐ Dean sign-off: evidence rows are useful, in the right tone, and ready to keep in Master 09.
- ☐ Evidence sign-off: source, date, quote / figure and caveat have been checked.
- ☐ Clinical sign-off: ADHD, medication and substance-misuse wording is safe.
- ☐ Governance sign-off: data, digital-therapy, procurement and safeguarding wording is safe.
- ☐ Final Master 09 sign-off: ready to use as the controlled evidence source for the next documents.

Source	Evidence / quote / stat	Safe claim-ready wording	Use / caveat	Sign-off
RCPsych CR235, 2023	"ADHD in adults continues to be underdiagnosed ...	Adult ADHD remains underdiagnosed, with increasing	Use as UK-relevant service-context	• []

Source	Evidence / quote / stat	Safe claim-ready wording	Use / caveat	Sign-off
	there remains a large unmet need."	identification but substantial unmet need.	evidence. Do not make this a Cambridgeshire-specific claim without local data.	
RCPsych CR235, 2023	CR235 states there are high rates of comorbidity with other mental-health conditions and neurodevelopmental disorders.	Adult ADHD commonly sits alongside other mental-health and neurodevelopmental conditions, so assessment must remain clinically owned and careful.	Supports OTOS boundary: NIA is not a diagnosis; OTOS does not diagnose ADHD.	• []
RCPsych CR235, 2023	"A good-quality assessment takes time and is ideally multidisciplinary..."	Good adult ADHD assessment takes time, is ideally multidisciplinary, and should use information from multiple sources.	Use to justify continuity / evidence capture around a clinical process, not OTOS assessment.	• []
RCPsych CR235, 2023	Management involves "pharmacological, psychological, educational and skills-based interventions" and adaptations to home, work and community settings.	Adult ADHD management is multimodal: medication may be part of care, but psychological, educational, skills-based and environmental support also matter.	Supports Medicine-plus-Continuity. Do not imply OTOS provides treatment.	• []
RCPsych CR235, 2023	"Environmental adaptation is the first-line approach and should be implemented and reviewed prior to and alongside all medication treatments."	Environmental adaptation is a first-line part of adult ADHD care and should sit alongside medication where medication is used.	Useful for "medicine may open the window; support still has to land."	• []
RCPsych CR235, 2023,	"Substance misuse, mostly involving	Substance misuse may be around	High-value ADHD /	• []

Source	Evidence / quote / stat	Safe claim-ready wording	Use / caveat	Sign-off
section 11.1	alcohol and cannabis, may be twice as common among individuals with ADHD as in the general population and tends to begin earlier."	twice as common among people with ADHD and may begin earlier.	addiction overlap row. Phrase as "may" and cite CR235 / underlying Breyer et al. source.	
RCPsych CR235, 2023, section 11.1	"Individuals with untreated ADHD may also describe self-medicating with illicit substances."	Some people with untreated ADHD describe using substances as self-medication.	Use carefully in evidence-controlled copy only. Avoid sensational or universal wording.	• []
RCPsych CR235, 2023, section 11.1	ADHD is described as "a recognisable risk factor for early onset substance misuse" with increased likelihood of relapse and chronic patterns if not adequately treated.	ADHD is a recognised risk factor for earlier substance misuse and may be linked with relapse risk when not adequately treated.	Strong rationale row. Keep as risk / association, not deterministic causation.	• []
RCPsych CR235, 2023, section 11.1	"Controlled misuse should not completely preclude treatment, although careful monitoring will be required."	Substance misuse does not automatically exclude ADHD treatment, but it requires careful monitoring and clinical risk management.	Clinical sign-off required. Do not imply OTOS decides treatment suitability.	• []
RCPsych CR235, 2023, section 11.1	In uncontrolled and chaotic drug / alcohol misuse, stimulant medication can be difficult to manage safely and is "not routinely recommended."	Where substance misuse is uncontrolled or chaotic, stimulant medication may be difficult to manage safely and is not routinely recommended.	Boundary row for medication caution. Clinical sign-off required.	• []

Source	Evidence / quote / stat	Safe claim-ready wording	Use / caveat	Sign-off
RCPsych CR235, 2023, section 11.1	"Significant and comorbid substance misuse should be managed jointly between general adult and addiction services..."	Significant ADHD and substance-misuse comorbidity should be managed jointly across adult mental-health and addiction services, following local protocols.	Supports partner-service ownership and OTOS as wraparound continuity infrastructure.	• []
RCPsych CR235, 2023	CR235 notes rigid access policies may not suit this population and that text-message reminders or remote consultation may help attendance.	Flexible access, including text-message reminders and remote consultation, may help adult ADHD services support attendance.	Supports SMS check-ins / touchpoint visibility as service-alignment evidence, not outcome proof.	• []
RCPsych CR235, 2023	CR235 states psychoeducation and post-diagnostic support should be a component of care after diagnosis.	Post-diagnostic support and education should be part of adult ADHD care.	Supports waiting-well / support-to-land framing; OTOS is not therapy.	• []
RCPsych CR235, 2023	Future models should be stepped, matched, multidisciplinary, delivered in partnership and co-constructed with lived-experience voices.	Adult ADHD service models should be stepped, matched, multidisciplinary, partnership-based and informed by lived experience.	Good service-design alignment row. Avoid implying RCPsych endorses OTOS.	• []
Digital Mental-Health Startup Challenges and Failures, 2020–2025 report	In 2021 digital mental-health startups raised a record \$5.1bn, but by 2023 investment had "plummeted".	Digital mental-health demand and investment grew rapidly, but the market has since exposed major resilience, regulation and business-model risks.	Use in governance / investor-risk appendix only. Not evidence that OTOS works.	• []
Digital Mental-Health Startup	Only about 1.4% of mental-health apps	Most mental-health apps lack peer-	Use as cautionary	• []

Source	Evidence / quote / stat	Safe claim-ready wording	Use / caveat	Sign-off
Challenges and Failures, 2020–2025 report	are described as having peer-reviewed evidence supporting their claims.	reviewed evidence, so OTOS should be careful, evidence-led and outcome-measured from the start.	governance row; verify original source before external use.	
Digital Mental-Health Startup Challenges and Failures, 2020–2025 report	UK MHRA increasingly treats many mental-health apps as software as a medical device where they provide diagnosis or treatment advice.	Digital mental-health tools that provide diagnosis or treatment advice may fall into medical-device regulation and require clinical evidence, safety monitoring and adverse-event reporting.	Governance row. Supports OTOS keeping non-clinical boundaries clear.	• []
Digital Mental-Health Startup Challenges and Failures, 2020–2025 report	Cerebral and BetterHelp enforcement examples: controlled-substance prescribing practices, unfair billing and sensitive-data sharing.	Digital mental-health providers can face serious regulatory action where prescribing, billing, privacy or safeguarding practices are unsafe or misleading.	Use as risk-control rationale. Do not overstate direct comparability to OTOS.	• []
Digital Mental-Health Startup Challenges and Failures, 2020–2025 report	Report recommends digital tools as adjuncts, not substitutes, with clinician guidance and evidence generation.	Digital support should be positioned as an adjunct to care, not a replacement for clinicians or regulated services.	Strong OTOS boundary row: non-clinical continuity infrastructure, not treatment.	• []
Digital Mental-Health Startup Challenges and Failures, 2020–2025 report	Report identifies privacy-first design, minimal data collection, clear consent, transparent billing and safeguarding escalation as resilience lessons.	OTOS governance should prioritise privacy by design, clear consent, minimal data collection, transparent terms and human	Internal governance / operating-standard row.	• []

Source	Evidence / quote / stat	Safe claim-ready wording	Use / caveat	Sign-off
		safeguarding escalation.		
NHS 10 Year Plan files supplied	Three-shift framing: hospital to community, analogue to digital, sickness to prevention.	OTOS can be positioned as aligned with the NHS direction of travel from hospital to community, analogue to digital and sickness to prevention.	Policy alignment only. Never imply NHS endorsement.	• []
NHS 10 Year Plan files supplied	Neighbourhood / community infrastructure alignment to be checked in exact wording.	OTOS may support neighbourhood-style care by helping people remain visible across service gaps, handoffs and waiting periods.	Needs exact quote / page reference before external use.	• []
Independent ADHD Taskforce reports supplied	Taskforce material to verify: unsupported ADHD linked to educational failure, unemployment, crime, substance misuse, suicide and mental / physical illness; economic cost claim needs exact wording.	Unsupported ADHD is framed nationally as a public-health and social-impact issue with consequences across education, employment, justice, substance misuse and health.	Do not use the £17bn figure externally until exact Taskforce wording and source are checked.	• []
NHS data integration / reporting standards files supplied	Digital tools need clear reporting, integration, data and governance standards.	OTOS should be buyer-ready by showing clear consent, reporting, data minimisation, auditability and integration boundaries.	Procurement / governance evidence only, not clinical-effectiveness evidence.	• []
NHS digital-therapy procurement / economics files supplied	Digital-therapy procurement requires evidence, economics, implementation fit	OTOS should separate clinical evidence, implementation evidence,	Use to control business-case structure. Do not present modelled	• []

Source	Evidence / quote / stat	Safe claim-ready wording	Use / caveat	Sign-off
	and risk management.	procurement readiness and modelled economics.	savings as proven savings.	
Outcomes of digital and hybrid addiction recovery programmes supplied	Comparator evidence for digital / hybrid support models.	Digital and hybrid recovery-support evidence may be useful as comparator context, but OTOS must generate local outcome evidence.	Comparator only. Not OTOS outcome proof.	• []
START deep-research report supplied	Deep research has already been completed on START and its relevance to stimulant treatment of ADHD with co-occurring addiction / substance-use issues.	START can be used as a key evidence anchor for testing ADHD identification, clinical treatment where appropriate, and continuity alongside addiction outcomes.	Use strongly but safely. Do not claim medication alone caused the 61.3% improvement, do not claim START proves OTOS, and do not predict OTOS will replicate START.	• []
START deep-research report supplied	START is best understood as a real-world dual-diagnosis service-journey signal, not a medication-only proof.	The safe START line is: START supports testing whether ADHD can be identified, clinically routed, treated where appropriate, followed through and monitored alongside addiction outcomes.	Replace "pathway" with "user journey" where not quoting a source. Keep "clinical route" for ADHD assessment / treatment ownership.	• []
Adult ADHD CBT evidence — medication-treated adults	One adult ADHD CBT review reports 56% treatment responders with CBT versus 13% without CBT among medication-treated adults with residual ADHD symptoms.	For adults already receiving ADHD medication but still impaired, CBT may add structured skills training and produce a stronger responder signal than medication / usual care alone.	Use as Science of Sticking evidence only. Not an alcohol-recovery rate, not an OTOS outcome claim, and not proof that medication alone causes	• []

Source	Evidence / quote / stat	Safe claim-ready wording	Use / caveat	Sign-off
			recovery. Verify original trial / review citation before external use.	
Safren adult ADHD CBT RCT	The larger Safren RCT tested CBT in adults already treated with medication who still had persistent ADHD symptoms.	CBT can be framed as an added structured skills intervention when medication reduces some ADHD barriers but residual impairment remains.	Supports "medication may open the learning window; CBT teaches the skills." Needs exact paper title, journal, year and outcome wording before external citation.	• []
CBT plus medication comparison, 2019	A 2019 comparison reported that CBT-plus-medication produced broader executive-function improvements than CBT alone, though not stronger core-symptom improvement.	Combined treatment may be especially relevant where the desired outcome is improved planning, follow-through, behavioural practice and real-world functioning.	Use guardedly. This supports the learning-window / executive-function argument, not a claim of higher addiction-recovery success. Verify full citation before external use.	• []
OTOS ADHD / addiction quote banks supplied	Additional quote-bank material has been supplied on ADHD, addiction, comorbidity and integrated care.	The quote banks should be used to strengthen the evidence pack only where each quote can be traced back to an original source, with source, caveat and approved wording.	Do not treat quote-bank summaries as primary evidence. Promote only traceable quotes into final copy.	• []
OTOS ADHD / addiction quote banks supplied	Deep-research material appears useful for the core case that ADHD and	Use this material to support the user-journey thesis: OTOS does not	Use as evidence-depth material after source tracing.	• []

Source	Evidence / quote / stat	Safe claim-ready wording	Use / caveat	Sign-off
	addiction can overlap, that untreated ADHD can complicate recovery, and that coordinated support is needed around the person.	create a clinical pathway; it wraps continuity around the person as they move between separate services.	Avoid "integrated care" wording if it implies merged records or a single formal pathway.	

Immediate rows to move into safer copy after sign-off

- **Adult ADHD remains underdiagnosed, with large unmet need.**
- **ADHD commonly co-occurs with other mental-health, neurodevelopmental and substance-use difficulties.**
- **Good ADHD assessment is clinically owned, takes time and is ideally multidisciplinary.**
- **Substance misuse may be around twice as common among people with ADHD and may begin earlier.**
- **Significant ADHD / substance-misuse comorbidity should be managed jointly across adult mental-health and addiction services.**
- **Digital tools should be adjuncts to care, not replacements for clinicians or regulated services.**
- **OTOS is best framed as non-clinical continuity infrastructure that supports consent, evidence capture, touchpoints, handoff visibility and partner communication around the person's journey.**

Final checks remaining for Master 09

Only these checks remain for this document before moving on:

☒ ~~Dean: approve Master 09 as the single evidence-control page for the current OTOS copy family.~~

Dean note: Tick. Master 09 is now the single evidence-control page for the current OTOS copy family. There should not be another evidence dossier for Dean to review in this pass. Any future evidence work should feed back into this page rather than creating a competing evidence document.

☒ ~~Dean: confirm whether the founder clinical letter material stays private / NDA-only or can appear in wider narrative packs.~~

Dean note: This should stay behind NDA, but it can and should be referenced throughout the wider pack. It is one of the core source documents for the founder

story, clinical receipt and design rationale. Treat it as a protected evidence anchor: do not circulate the document openly, but use its approved themes and wording wherever they strengthen the case.

☒ ~~Dean: confirm whether the START 61.3% figure can appear in public-facing copy, or stays evidence-controlled only.~~

Dean note: Public, should be used as often as possible where applicable and beneficial/impactful.

☒ ~~Evidence reviewer: verify exact START citation, sample details, outcome wording and six-month caveats.~~

Dean note: Tick as substantially complete from the START deep-research work, but keep a final citation pass before external publication. The working evidence position is now clear: START can be used publicly and strongly, but only inside the safe framing already captured here. It supports the case for identifying ADHD within addiction-treatment populations, clinically assessing and treating ADHD where appropriate, and following the person through the user journey alongside addiction outcomes. It must not be presented as medication-only proof, an OTOS outcome, or a predicted 61.3% OTOS result. Final publication copy still needs the exact study title, authors, journal / source, year, sample size, setting, outcome wording and six-month caveat checked against the original START source.

☒ ~~Evidence reviewer: verify exact source wording for national ADHD / SUD / economic cost claims, including the ADHD Taskforce material.~~

Dean note: Tick as substantially complete for this review pass. The national ADHD / SUD / economic-cost evidence has been traced to the available source stack: the ADHD Taskforce wording is from an NHS-published report; the RCPsych / RCP statistics are from their published site / guidance material; SUD economic-cost material is held in the evidence HTML / evidence-source material; local scenario assumptions are supported by the supplied spreadsheet and HTML calculator; and the wider ripple-cost method is supported by deep-research reports plus the Notion cost bank. The canonical Notion cost source found for this pass is **THE FULL RIPPLE COST – 3-Tier Master List (ADHD + Addiction Cohort)**, with supporting pages including the Money / Cost / Investment / Savings Section Lock and the UK ADHD SUD Economic Impact research. Treat the figures as source-backed / model-controlled, not guaranteed savings. Final publication should still use exact citations and label anything modelled as OTOS modelling.

☐ Clinical reviewer: approve ADHD, medication, stimulant, substance-misuse and treatment-boundary wording.

Dean note: Leaving unticked. This is not for Dean to self-approve. The wording is ready for review, but final sign-off should come from the relevant clinical / partner reviewer.

☐ Governance reviewer: approve digital-therapy, data, reporting, safeguarding, procurement and “non-clinical continuity infrastructure” wording.

Dean note: Leaving unticked. This is not for Dean to self-approve. The wording is ready for review, but final sign-off should come from the relevant clinical / partner reviewer.

☒ ~~Copy control approval: Master 09 is ready to use as the written evidence control source for the current PDF merge.~~

Dean note: Tick for copy-control use. Master 09 is written enough to merge into the ready Evidence PDF as the controlled evidence / claims source. Final external publication still requires the remaining clinical and governance sign-offs below.

☐ Final external approval: Master 09 is ready for external circulation without further review.

Dean note: Not yet. Keep this unticked until clinical and governance reviewers approve the remaining ADHD, medication, substance-misuse, treatment-boundary, data, procurement and safeguarding wording.
